

Our Ref: (T) /HC/ 0729851

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Rheumatology

WG 01, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS]. [POSTCODE]

[ADDRESS]

It was a pleasure to meet Mrs [NAME] today accompanied by her husband. I understand that her disease is currently being controlled with an elemental/refined diet though she remains very current weight 39.7kg. She informs me that she is no longer on a steroid treatment - she was of 15-20 mg of prednisolone equivalent for around seven years. I note that she has also been small bacterial overgrowth with rifaximin and ciprofloxacin.

In terms of her osteoporosis treatment, Mrs [NAME] received one of the three planned zoledron infusions, given in 2015. Her 2016 infusion was cancelled due to concerns of a flare of Crohn which led to a capsule endoscopy, though her symptoms are put down to small bacterial overgro most recent planned infusion in 2017 she cancelled due to planned dental treatment - she is d two crowns and one dental extraction. She is very concerned about media reports of osteonecra jaw. I informed her that this is extremely rare occurrence of incidence roughly 1 in10,000. safety precaution we generally recommend delaying treatment with a bisphosphonate to around s after the dental treatment. However Mrs [NAME] is very reluctant to continue this at present like to know how much of a benefit it has had on her bone health.

I note that her last DXA scan in 2016 showed T score of the AP spine of -1.7 which demonstrat improvement and T score left femoral neck -3.8, right femoral neck -3.3 which was stable.

As a consequence I have arranged for her to have bone turnover markers and parathyroid hormon assess bone turnover and I have arranged a DXA scan for summer 2018 which would be a year fro previous.

Mrs [NAME] also mentioned that she has been having increasing joint symptoms and is concerned developing a Crohn's-related arthritis. She describes pain and stiffness in her PIPs, both k ankles, the balls of her feet and also around the left Achilles tendon region. She does descr stiffness at her knees in the mornings lasting around two hours. She reports that her joints occasionally swollen though they are not currently.

On examination today there was no joint swelling and she had a good range of movement of all including knees though there was significant crepitus suggestive of patellofemoral osteoarthr had some tenderness of her left Achilles tendon but no evidence of swelling.

I expect these changes are likely degenerative in nature but given her history of Crohn's dis arranged for X-rays of her hands, feet, ankles and knees and will check her inflammatory mark rheumatoid factor anti-CCP. I have arranged a follow-up in a year's time following her DXA s point we can decide on her further osteoporosis treatment. Of course if any of her investigat an inflammatory arthritis I will arrange to see her sooner.

With many thanks and best wishes

Yours sincerely

Checked electronically and sent unsigned to avoid delay by:

Electronic signature will go here DO NOT REMOVE

[PROVIDER-NAME]

Specialist Registrar